

Sales & integration approach

Who we sell to, the message by buyer, and how we land and deploy without a rip-and-replace.

WHO WE SELL TO (IDEAL CUSTOMER)

- **Safety-net & community hospitals, DPC / private practices, and underserved / LMIC systems** — buyers worn down by cost, vendor lock-in, and clinician burnout.

THE MESSAGE, BY BUYER

CMO / CWO

Retention and Joy in Medicine; measurable burnout reduction; a real note, not homework.

CIO / CTO

Standards-native (FHIR/USCDI/TEFCA), composable, no lock-in, low total cost. Buy certified modules, don't rebuild.

CFO

Transparent published pricing, denial-prevention ROI, and price-transparency compliance handled.

Clinicians & nurses

Fewer clicks, ambient notes, gentle alerting — designed *with* nurses, the largest user group [PMID 21524315](#).

LAND WHERE LEGACY HURTS

- **Open the door on pain:** burnout, opaque expensive contracts, rigid workflows — then land a **low-risk pilot on one service line**, not a house-wide switch.

INTEGRATION APPROACH — HOW WE DEPLOY

- **Standards-native connectors:** eligibility & prior auth (Da Vinci), labs, e-prescribing (NCPDP SCRIPT), telehealth, and TEFCA exchange — interoperability that returns time.
- **Composition:** certified modules on a FHIR-native core; we integrate, we don't reinvent.
- **Gated go-live (Wave 0 → 3):** readiness → pilot → specialty expansion → enterprise, with super-users, training, and tiered support. A wave graduates on data — adoption & minutes-returned [PMID 33017724](#).

OBJECTIONS → ANSWERS

- **"You're a prototype."** → A phased, low-risk pilot; every feature is evidence-cited; a clear ONC certification path.
- **"Switching is too costly."** → Standards-based migration & two-way interoperability; run alongside, don't rip out.
- **"Who supports it?"** → A tiered support model and super-user program — support is a top adoption factor, and we resource it.